



SMHS Mental Health rights of carers other personal support persons

Policy code	<i>SMHS: 38</i>
Original endorsement date	<i>October 2015</i>
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Functional sub group	Mental Health
Summary	The South Metropolitan Health Service is committed to ensuring the rights of personal support persons – which includes parent/guardians, carers, close family members and nominated persons – are respected by the health service. This commitment includes ensuring personal support persons are made aware of their rights under the <i>Mental Health Act 2014</i> .
Policy owner	Executive Director Area Mental Health
Contact	Carole Steiner
Review date	<i>August 2028</i>
NSQHS Standards	Standard 1: Clinical Governance Standard 2: Partnering with Consumers Standard 5: Comprehensive Care Standard 6: Communicating for safety
Chief Psychiatrists Standards for Clinical Care	Standard 2: Assessment Standard 3: Care planning Standard 4: Consumer and carer involvement in individual care Standard 6: Risk Assessment and management Standard 7: Transfer of Care
Status	<i>Active</i>

1. Background

1.1. About this document

The South Metropolitan Health Service (SMHS) is committed to providing information and the inclusion of carers and other support persons in providing care. Carers and other support persons should be key partners in delivery of ongoing care to consumers. This commitment includes ensuring personal support persons are made aware of their rights under the [Mental Health Act 2014 \(MHA 2014\)](#).

1.2. Key definitions

Term	Definition
Carer	Defined by Carers Recognition Act 2004 , a carer refers to an individual who provides ongoing care, support and assistance to a person with disability, a chronic illness (which includes mental illness) or who is frail, without receiving a salary or wage for the care provided.
Close family member	Defined by the MHA 2014 s. 281 as a family member who: • is not also the persons carer or the persons nominated person but • provides ongoing care or assistance to the person May be any of the following persons spouse, de facto partner, a child, stepchild, a parent, stepparent, foster parent, a sibling, grandparent, aunt or uncle, niece or nephew or cousin. If the person is of Aboriginal or Torres Strait Islander descent any person regarded under the customary law, tradition or kinship of that person's community is considered as the equivalent of close family member.
Enduring guardian	A person appointed under an Enduring Power of Guardianship to make personal, lifestyle and treatment decisions on behalf of the adult person who makes the appointment. The Guardianship and Administration Act 1990 Part 9A.
Enduring power of Guardianship (EPG)	A formal document in which a person nominates a competent adult to be an Enduring Guardian. An EPG authorises a person to make important personal, lifestyle and treatment decisions on behalf of the patient once incapable of making such decisions. This person is known as an Enduring Guardian. To make an EPG you must: • be 18 years of age or older, • have full legal capacity • be made voluntarily and without inducement or coercion, and • be witnessed by two people (one of whom must be an authorised witness). The scope of authority given to the EPG is determined by the patient when making an EPG.

Nominated person	Someone who is formally nominated by person experiencing mental illness. The nominated person can be any adult the patient chooses MHA 2014 s. 274.
Parent or guardian of a child	The person who has responsibility for the child as defined in the Family Court Act 1997
Personal support person	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, nominated person; the parent or guardian of a child and any guardian or enduring guardian of an adult MHA 2014 .

2. Policy Statement

The [MHA 2014](#) provides that the role of carers and other support persons in the provision of treatment, care and support to a person who has a mental illness should be acknowledged and respected.

Carers and/or other support persons must be contacted as soon as possible and be informed with respect to the patients care and treatment.

2.1. Rights of nominated persons

Unless the patient's psychiatrist reasonably believes it is not in the patient's best interests ([refer to Section 5](#)), a patient's nominated person is entitled to be provided with information about the patient's treatment and care, including information about the following matters:

- the mental illness for which the patient is being provided with treatment or care
- the grounds on which any involuntary treatment order was made in respect of the patient
- the treatment and care proposed to be provided to the patient
- any other options for the patient's treatment and care that are reasonably available
- the treatment provided to the patient and the patient's response to that treatment
- the seclusion of the patient
- the use of bodily restraint on the patient
- the types of services available, including support services that may meet the patient's needs.

Unless the patient's psychiatrist reasonably believes it is not in the patient's best interests, a patient's nominated person is entitled to be involved in matters relating to the patient's treatment and care including:

- the consideration of the options that are reasonably available for the patient's treatment and care
- the provision of support to the patient; and
- the preparation and review of any Treatment, Support and Discharge Plan for the patient.

Refer to [MHA 2014](#) s. 266.

A patient's nominated person is entitled to be provided with information about the patient's rights under the [MHA 2014](#) and how those rights can be accessed and exercised.

Unless authorised to do so in another capacity, a patient's nominated person is not authorised to apply on behalf of the patient for admission or discharge by a mental health service or to make a treatment decision about the provision of treatment to the patient.

2.2. Rights of carers and other support persons

Any carer or other support person of a patient must be provided with information about the patient and carer's or other support persons rights under the [MHA 2014](#) and how those rights can be accessed and exercised.

Where a patient has more than one carer or support persons then it is sufficient compliance with the [MHA 2014](#) if at least one carer or support person is involved.

A patient's carer and other support persons have the right to ask for and be given general verbal and written information about:

- The mental health illness and diagnosis.
- What may be expected and how to manage.
- Medication - alternatives, benefits and possible side effects.
- Available local services for the consumer - both inpatient and community.
- Available local services for the carer – government, private and community.
- Local and national support groups.

Carers and other support persons should be given:

- Ongoing opportunities to ask questions, give feedback, and discuss concerns.
- The opportunity to see health clinicians on their own.
- The right to confidentiality in communications with and to a clinician.
- Positive feedback as a valued member of the care team.
- Confidence to express their opinion and concerns.
- Practical and emotional support

Subject to any limitations regarding consent ([refer to Section 4](#)), any carer or other support person of a patient is entitled to be provided with information about the patient's treatment and care, including information about the following matters:

- the mental illness for which the patient is being provided with treatment or care
- the grounds on which any involuntary treatment order was made in respect of the patient
- the treatment and care proposed to be provided to the patient
- any other options for the patient's treatment and care that are reasonably available
- the treatment provided to the patient and the patient's response to that treatment
- the seclusion of the patient
- the use of bodily restraint on the patient; and
- the types of services available, including support services that may meet the patient's needs.

Refer to [MHA 2014](#) s. 285.

Subject to any limitations regarding consent, any carer or other support person of a patient is entitled to be involved in:

- the consideration of the options that are reasonably available for the patient's treatment and care
- the provision of support to the patient and
- the preparation and review of any Treatment, Support and Discharge Plan for the patient.

Unless authorised to do so in another capacity, any carer or close family member of a patient is not authorised to apply on behalf of the patient for admission or discharge by a mental health service or to make a treatment decision about the provision of treatment to the patient.

3. Roles and responsibilities

3.1. Managers/Supervisors

Managers have the responsibility to:

- Include the rights of carers and other support persons in staff orientation and ongoing education programs
- Ensure sufficient information (e.g. posters, pamphlets, website, welcome letters) are available for staff to provide to patients, carers and other support persons.
- Ensure staff ask the patient if they have a carer or other support person

3.2. Staff

All SMHS mental health staff have the following responsibilities:

- Ensure that when being admitted or received, a person is asked whether they have a carer or other support person, and whether they consent to those persons being provided with information about or involved in the treatment and care of the person. Refer to [MHA 2014 s. 296](#).
- Provide information to carers and other support persons in appropriate formats for those with cultural and linguistically diverse needs. Evidence of these actions must be documented in the medical record.
- Ensure information exchanges between the service, patient's carers and other support persons are age appropriate and use a variety of methods and media to assist patients, carers and other supports persons to understand their rights.
- Offer the use of interpreters when indicated
- Provide opportunities to ask questions
- Engage with patient advocates and involve carers or support persons in the discussion of rights and responsibilities.
- Ensure patients, carers and other support persons are informed of and supported in easily accessing feedback and complaints processes
- The person responsible providing a patient with an explanation of their rights must also ensure that the carer or other support person of the patient is provided with an explanation of the patient's rights, regardless of whether the patient has consented to the carer or other support person being provided with that explanation of rights.

4. Consent to sharing information and/or involving carers or other support persons

A person who consents to a carer or other support person being provided with information about or being involved in the treatment and care of the person can withdraw consent at any time; likewise, a person who refuses consent can consent at any time. Refer to [MHA 2014](#) s. 298.

If the person does not consent, or the consent is given but later withdrawn, the health service must ensure that the person is periodically asked during their admission whether they consent to the information being provided to their carer or other support person. [MHA 2014](#) s.297 (2)

Health services must ensure that it is documented in the medical record each time the person is periodically asked to give consent, and the person's answers at that time.

Whether a carer or other support person is entitled to be provided with information about the patient's treatment and care, or to be involved in matters relating to the patient's treatment and care, is dependent upon:

- the status of the person under the MHA 2014; and
- the capacity of the patient to consent.

MHA 2014 Status	Capacity to consent	No capacity to consent
Voluntary patient	Carer / close family member has right to information or involvement, with patient consent	Carer / close family member has right to information or involvement, unless considered not in patient's best interests
Involuntary patient or detainee under the Criminal Law (Mental Impairment) Act 2023	Carer / close family member has right to information or involvement, unless patient refuses consent, and such refusal is reasonable	Carer / close family member has right to information or involvement, unless considered not in patient's best interests

Refer to [MHA 2014](#) s. 286-289.

The person responsible for providing any carer or other support person with information or involving any carer or other support person in a matter, shall continue to make reasonable efforts to do so until it is reasonable to conclude that it is not possible to provide any carer or other support person with the information or involve them in the matter. Refer to [MHA 2014](#) s. 268 & 291.

5. Persons best interests

In deciding what is or is not in the best interests of a person, health services must consider:

- the wishes of the person
- the views of the person's enduring guardian or guardian, parent or guardian (if the person is a child), nominated person, carer and close family member; and
- any other matter that the health service considers relevant to making the decision. Refer to [MHA 2014](#) s. 7.

If a patient's psychiatrist reasonably believes a carer or other support person is not entitled to be provided with particular information or involved in a particular matter because it is not in the patient's best interests, he/she must advise the carer or other support person of the decision and the reasons for it. This advice and the reasons for it must be documented in the patient medical record and a copy given to the patient and Chief Mental Health Advocate.

If the carer or other support person requests written confirmation of the advice, the psychiatrist will provide the written confirmation, document in the patient's medical record and give a copy to the patient. Refer to [MHA 2014](#) s. 292.

The carer or other support person will be informed of their right to make a complaint and provided with information on how to lodge a complaint and the process involved. A complaint may be made directly to the mental health service or to the Health and Disability Services Complaints Office (HaDSCO).

A psychiatrist may revoke a decision that a carer or other support person is not entitled to be provided with information or involved in a matter. If the carer or other support person previously requested to be provided with the information or involved in the matter, the psychiatrist must ensure that this is done as soon as practicable and document the provision of the information in the patient's medical record and give a copy to the patient. Refer to [MHA 2014](#) s. 294.

6. Compliance monitoring

6.1. Compliance monitoring

SMHS MH Site Service Co Directors are responsible for ensuring staff in their areas are made aware of and comply with this policy, and that compliance risks and training requirements are managed.

SMHS MH contributes to an annual report demonstrating compliance with the [Carers Recognition Act 2004](#) facilitated by SMHS Safety Quality and Consumer Engagement Unit.

7. Legislation

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

- [Carers Recognition Act 2004](#)
- [Mental Health Act 2014](#)
- [Guardianship and Administration Act 1990](#)
- [Family Court Act 1997](#)
- [Criminal Law \(Mental Impairment\) Act 2023](#)
- [Health and Disability Services \(Complaints\) Act 1995.](#)

8.Related Documents

8.1. WA Health mandatory policies.

- [Patient Confidentiality Factsheet](#)
- [Language Services Policy](#)
- [Language Services Procedure](#)

8.2. SMHS policies

- [Rights of Patients in Mental Health](#)
- [Carers](#)

8.3. Other documents

- [Charter of Mental Health Care Principles](#)

8.4. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP 3.0	28 August 2025	15 August 2025	15 August 2028	August 2028	Triennial Review

6.6. Approval

Approval by	Executive Director Area Mental Health
Approval date	15 August 2025

This document can be made available in alternative formats on request.

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